

Master 05 — Partner Pack

Audience: Local partner services and team leads.

Purpose: Give partners a practical, low-burden explanation of what participation could mean and what it does not mean.

Status: DRAFT — needs Dean review.

Priority: P1.

Review needed from: Dean; partner reviewers; governance / IG reviewer.

Source basis: Master 05 source text, Programme Lock shared-thread section, START update, Programme Plan partner model.

A4 REVIEW PAGE 1 — Partner pack opening

OTOS Continuity™ is being prepared as the non-clinical infrastructure around a specific intervention test.

The intervention tests whether the OTOS Continuity Management Layer and user Continuity Thread can support a suspected ADHD-driven addiction cohort:

- identify adults whose addiction, relapse or repeated crisis may be driven by untreated or inadequately treated ADHD;
- route the ADHD driver toward qualified clinical assessment and treatment where clinically indicated;
- provide a non-clinical Continuity Management Layer through which authorised partners can manage their agreed part of continuity;
- follow the user's whole-person journey over time through a Continuity Thread built from agreed non-clinical events;
- enable authorised partners to support recovery work, addiction support, CBT, structured support / waiting-well routes, service handoffs, behavioural reinforcement and habit-stacking practice to land;
- evidence what happened;
- decide whether to continue / continue with changes / rebuild / pause / stop.

The first proposed test is bounded: up to 50 adults, 12 weeks, Cambridge / Cambridgeshire and Peterborough context, subject to governance.

The ask is not to endorse a finished platform.

The ask is to help test whether this suspected ADHD-driven addiction cohort can be found, clinically routed, supported through authorised partner activity in the Continuity Management Layer, followed through a whole-person Continuity Thread, and evidenced safely.

A4 REVIEW PAGE 2 — What partners are not being asked to do

A partner is not being asked to:

- give OTOS access to clinical notes;
- merge records;
- create a shared clinical record;
- change clinical responsibility;
- let OTOS diagnose ADHD;
- let OTOS prescribe or manage medication;
- let OTOS treat addiction;
- let OTOS deliver therapy;
- let OTOS provide crisis care;
- let OTOS make safeguarding decisions;
- let OTOS automate clinical triage;
- make public outcome claims;
- commit to rollout before local evidence exists.

Partner services keep their own responsibilities.

Clinical teams make clinical decisions.

OTOS surfaces continuity signals.

A4 REVIEW PAGE 3 — Shared continuity infrastructure, not shared clinical record

OTOS does not ask partners to merge records or give other services access to clinical notes.

The model is:

One person. One Continuity Management Layer. One Continuity Thread following the user journey. Separate service records. Authorised partner access only.

In simple terms:

- **Layer** = the non-clinical infrastructure partners and the user access.
- **Thread** = the user's whole-person journey over time, built from agreed non-clinical events.
- **Service lines** = each partner's own records, communication channels, service work and responsibilities.

Where consent, role and governance allow, the user's Continuity Thread may contain agreed continuity signals:

- touchpoints;
- handoff receipts;
- SMS delivery / failure / reply / silence;
- attendance or engagement status;
- ADHD clinical-route status / next-step status where agreed;
- medication-status category where agreed;
- drift / silence signals;
- human-review status;
- evidence-export fields.

The thread must not contain:

- clinical notes;
- therapy notes;
- prescribing instructions;
- diagnostic records;
- full service records;

- EPR content;
- private session content.

Partner teams keep their own service records, communication channels, service work and responsibilities. The Continuity Management Layer is shared continuity infrastructure, not a shared clinical record. The Continuity Thread follows the user journey through agreed non-clinical events.

A4 REVIEW PAGE 4 — What a partner may be asked to do

A participating partner may be asked to:

1. nominate a named contact;
2. agree simple entry / handoff rules;
3. identify where the suspected ADHD-driven addiction pattern may appear;
4. support consent-led onboarding where agreed;
5. log or confirm agreed touchpoints;
6. receipt whether a handoff landed;
7. contribute simple support, attendance or agreed Thread-event signals where agreed;
8. review drift / silence items where appropriate;
9. help interpret the Week-12 evidence.

Minimum viable participation should be deliberately low-burden.

No partner should be listed as operationally active until the role is agreed.

No partner should be asked to carry risk it has not accepted.

A4 REVIEW PAGE 5 — Evidence posture for partners

The first test produces decision-grade local evidence, not national proof.

START helps justify the test. It is a real-world dual-diagnosis service-journey signal, not an OTOS outcome and not a medication-only proof. OTOS must generate its own local evidence.

The partner evidence question is practical:

- Can the cohort be identified safely?
- Can the next step be made visible through agreed Layer activity and authorised Thread events?
- Can partner-approved micro mobile check-ins keep contact warm?
- Can silence be noticed earlier?
- Can handoffs be receipted?
- Can human review operate safely?
- Can partner burden remain tolerable?
- Can the Week-12 evidence pack support the formal decision set using continuity signals and service-journey activity, not clinical outcomes or full partner records?

The answer may be continue, continue with changes, rebuild, pause or stop.

That is the point of the test.

Review notes

Dean decision needed

☒ ~~Confirm whether this Partner Pack should stay generic or be split into route-specific packs only.~~

Dean note: Keep this as the generic partner pack, with route-specific briefs used where the audience needs more detail.

☒ ~~Confirm whether START should appear in the generic partner pack or be left to evidence-controlled docs.~~

Dean note: START can appear if useful, but only in the safe Master 09 framing: real-world dual-diagnosis service-journey signal, not medication-only proof, not an OTOS outcome and not a predicted 61.3% result.

☒ ~~Confirm whether "up to 50 adults" appears here or only in Master 07.~~

Dean note: "Up to 50 adults" can appear here because partners need to understand the bounded, low-burden scale of the first test.

Evidence / citation check needed

☒ ~~START citation if included in external version.~~

Dean note: Tick in principle. If START remains in the external version, final publication copy must insert the exact citation and Master 09 caveats.

Clinical / governance check needed

- ☐ IG review of thread-content wording.

Dean note: Leave unticked for now. This needs IG / DPO / governance input before approval.

- ☐ Partner review of low-burden participation list.

Dean note: Leave unticked for now. This needs partner input before approval.

Copy approval

- ☐ Approved
- ☐ Needs edit
- ☐ Hold
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